

VYTALIX

MaternaLink partnership proposal

Co-designing the first evaluation · in development

THE PROBLEM YOU ALREADY OWN

The problem you already own

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- UK: inquiries into maternity services (Ockenden 2022; Kirkup 2022) repeatedly found failures to listen to women, to escalate, and to hand over between teams. Persistent disparities by ethnicity and deprivation are reported by MBRRACE-UK (cite the current report before use).
- Nigeria: the largest absolute maternal-death burden in the world (WHO/UNICEF estimates; cite the current edition); the "three delays" — deciding to seek care, reaching care, receiving adequate care.
- Common to both: a woman's own account is under-used; information is lost at handover; education arrives in the wrong language; programmes cannot see who has fallen out of contact.
- Existing maternity records are record systems, not relationship systems. The gap is the communication and coordination layer.

Visual: two columns UK / Nigeria; monoline "gap" diagram; source line at 16px listing the reports; no statistic without its citation on the slide. ---



WHAT MATERNALINK (IN DEVELOPMENT) IS INTENDED TO BE

What MaternaLink (in development) is intended to be

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- Intended use (PROPOSED; TO VALIDATE with regulatory counsel): a software platform intended to support communication, education, appointment scheduling and information handover between women receiving maternity care and their care teams, and to support programme administration. It is not intended to diagnose, prevent, monitor, predict, prognose, treat or alleviate any condition, nor to provide clinical decision support.
- v1 features: enrolment and granular consent
- personalised contact schedule and reminders
- clinically signed-off education by gestation, language and literacy
- secure asynchronous messaging with a named owner and acknowledgement SLA
- self-recorded diary displayed, not interpreted
- care-team console
- structured handover summary
- single-entry escalation log

Visual: intended-use statement in a Mist-tinted box at the top; features in two columns below; persistent tag "Non-diagnostic — in development" in Ember. ---



Where we are, honestly

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- Exists today: a concept; an early prototype dashboard of unknown maturity; a re-based v1 scope; a PRD, MVP specification and roadmap (`/product/`).
- Does not exist today: a deployed product; any real-world data; a clinical partner; ethics approval; regulatory classification; a Clinical Safety Officer; a signed IP agreement.
- Four gates we are closing before anything goes live: (1) IP ownership documented
- (2) prototype audited for code ownership, security and data
- (3) Clinical Safety Officer appointed and DCB0129 hazard log opened
- (4) DPIA and lawful-basis analysis complete (UK GDPR; Nigeria Data Protection Act 2023).
- Design targets, not claims: DTAC-ready pack; DSPT "Standards Met"; Cyber Essentials Plus; penetration test before go-live; UK-region hosting with a Nigeria deployment cell option.

Visual: two-column "Exists / Does not exist"; four gate markers in Ember beneath; design targets in Slate. ---



WHAT A PILOT LOOKS LIKE (THE SHAPE, BEFORE THE ASK)

What a pilot looks like (the shape, before the ask)

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- UK: a 3–6-month service evaluation with one community midwifery team, under DCB0160 on the trust side, with a defined cohort (ESTIMATE $n \approx 50$ –150), no fee or nominal fee; Vytalix provides the safety case, DPIA template, training and support. The EPR remains the record; MaternaLink provides pasteable summaries; no integration in v1.
- Nigeria: a 12-month pilot in 1–2 named LGAs with an implementing partner's existing cohort (ESTIMATE 500–2,000 women), two-way SMS and defaulter tracing, DHIS2-compatible exports, data owned by the state or programme, a defined exit. Programme envelope under £0.5m/year; platform layer priced at £6–£40 per woman per year (ESTIMATE), SMS at pass-through.
- Both: a co-authored evaluation protocol; operational metrics only (contact completion, acknowledgement times, defaulters traced, reach); no outcome claims without a formal evaluation.
- Success metrics are design targets (`/docs/04\_MATERNALINK\_PRODUCT\_STRATEGY.md` §24), reported honestly per period.

Visual: two pilot cards UK / Nigeria; a timeline bar under each; Ember marker at "evaluation report". ---



For NHS trusts and LMNS (show to NHS audiences)

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- What we offer: a non-diagnostic tool that maps to actions you already own — Ockenden immediate and essential actions on listening and escalation; CNST Maternity Incentive Scheme safety actions (current year's actions TO VALIDATE); complaints about communication. Artefacts in the first meeting: intended-use statement, DCB0129 hazard log, DPIA template, DTAC readiness pack, penetration-test summary once complete.
- What we ask: a digital midwife or Head of Midwifery sponsor; a Clinical Safety Officer for DCB0160; one community team; a 3–6-month service-evaluation window; a letter of intent first, then an evaluation agreement.
- What we will not do: touch the EPR in v1; process data before the DPIA and Data Processing Agreement are signed; make any clinical or outcome claim; describe you as a "partner" or "customer" without written consent.
- Procurement: no purchase obligation; any later contract via G-Cloud or direct award under threshold (routes TO VALIDATE).

Visual: three-row "Offer / Ask / Will not" layout; NHS terminology kept plain; no NHS logo. ---



For NGOs, implementing partners and donors (show to Nigeria/Africa audiences)

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- What we offer: enrolment and follow-up tracking for your cohort; two-way SMS at scale without your team typing messages; defaulter-tracing lists for CHEWs; logframe-ready indicators and DHIS2-compatible exports; Yoruba content subset first, Hausa/Igbo/Pidgin on the roadmap; data owned by you or the state; local-hosting option (Nigerian data centre or nearby cloud region, TO VALIDATE).
- What we ask: an existing cohort in 1–2 LGAs; CHEW supervision and training capacity; a programme budget line or a joint grant application (Grand Challenges, UNFPA innovation calls, FCDO-funded programmes, Innovate UK/NIHR global health where a UK institution leads — eligibility TO VALIDATE); co-authorship of the evaluation protocol; NHREC/state ethics approval for any evaluation.
- What we will not do: arrive with a state-wide proposal; compete on price with an incumbent programme (we are aware of mDoc's Digital Mom Project in Ekiti and Lagos and will explore complementarity first); share individual-level data with any insurer or third party.

Visual: same three-row layout; a small SMS-flow diagram (enrol → remind → reply → defaulter list → CHEW visit). ---

in cohort size (ESTIMATE), or per-woman platform pricing, plus SMS pass-through; training and devices priced separately or by partners.



For universities and research groups (show to academic audiences)

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- What we offer: a real-world care-coordination deployment to study; co-designed evaluation protocols; shared authorship; a defined, later research track (v3) on multimodal risk signals that we will only pursue with a data partnership, HRA/REC (UK) or NHREC (Nigeria) approval, pre-registered protocols, TRIPOD+AI-style reporting and fairness analysis by ethnicity and deprivation.
- What we ask: methodological partnership for the UK service evaluation and feasibility study (2027–28); a data-sharing conversation for the research track only when v1 evidence exists; joint grant applications where a UK institution leads (NIHR, Innovate UK, Wellcome, Grand Challenges — eligibility TO VALIDATE).
- What we will not do: train any model on patient data without approvals; make predictive claims before prospective validation and a UK MDR / MHRA pathway; treat the research track as a product commitment (ESTIMATE 24–36 months and £1.5m–£4m to do properly).

• Institutions we intend to approach are named in `docs/04\_MATERNALINK\_PRODUCT\_STRATEGY.md` §10 as institutions,

Visual: timeline: service evaluation → feasibility study → formal evaluation → research track (dotted); gate markers at ethics and data-sharing points. ---



For telecom and messaging partners (show to telecom/gateway audiences)

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- What we offer: a maternal-health messaging and coordination use case with programme and NGO buyers; content-neutral SMS on shared phones (safeguarding by design); voice reminders in local languages on the roadmap; a data-residency-aware architecture; a white-label option for health propositions later (ESTIMATE set-up £25k–£75k plus £2–£8 per user per year; TO VALIDATE).
- What we ask: gateway access and pricing (SMS, voice, USSD, WhatsApp Business), sender-ID and DND compliance guidance (NCC rules TO VALIDATE), zero-rating or bundled data for enrolled women where possible, and a named contact for a pilot.
- What we will not do: pass identifiable health data to a telecom partner; send diagnosis or pregnancy status in SMS without explicit opt-in; commit volumes before a funded pilot exists.
- Working assumptions to test together: ~60 SMS per woman per year; ₦4–₦6 per SMS (TO VALIDATE current rates); cost per woman per year for messaging well under \$1 (ESTIMATE)

Visual: architecture strip: app/PWA — gateway — feature phone; lock icon on the data boundary; cost assumptions in a small table labelled ESTIMATE. ---



NEXT STEP

Next step

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- One specific step, one date: [NHS: 45-minute scoping call with the digital midwife and CSO]
- [NGO: cohort and budget-line conversation; joint grant outline]
- [University: methods meeting on the evaluation protocol]
- [Telecom: pricing sheet and sender-ID guidance].
- Then: letter of intent or MoU (described as such, never as a partnership) → pilot or evaluation agreement with DPA → kick-off after the four gates close.
- What you can expect from us: the intended-use statement, hazard-log starter, DPIA template and evaluation-protocol outline within one week of this meeting.
- Contact: [founder email]
- [LinkedIn]
- [website]

Visual: Deep background; partner-type tabs; Ember CTA style on the date line; footer "Vytalix — Technology for Life". --- ## Using this deck | Rule | Detail | |---|---| | One partner slide per room | Show slide 6, 7, 8 or 9 — not all four | | Gates before asks | Slide

